

I focus on claims of mental illness. Things like: *Your brain is not supposed to work like that*. My recent work is on the clarification and implementation of values in the diagnosis of addiction.

DISSERTATION WORK

Chapter 1

In this first chapter, I explain why the separation of “addiction” from “dependence” is not merely a verbal dispute, but is a diagnostically meaningful distinction. The word “addiction” (formally known as “substance use disorder” or “SUD”) receives this misleading interpretation mostly from the public, but the debate in the literature is recent enough that a proper checkpoint and review is due. What the literature is transitioning away from, and what the public often translates SUD to mean, is something like: *You’re unable to stop doing X*. Usually, this “doing X” is “using drugs” or “drinking alcohol.” Then they associate the SUD with “dependence.” *She is alcohol dependent*. In my research, I explain that the association between dependence and SUD is a conflation that has caused unacceptable misunderstandings in how each term is characterized in both social and clinical spheres. To offer clarity, I show precisely how an evaluatively *negative* interpretation of the word “dependence,” one focused on subjugation and compulsion, was the culprit causing most of this confusion. So, I explain how transitioning from an evaluatively negative interpretation of dependence to a *strictly teleological one* leaves us with a more precise and accurate definition of SUD. That is, when the success of a dependency is teleologically framed, the evaluation of the goal(s) in question—as in calling them good or bad goals—become independent of the dependency’s pathologization. In other words, it is not the dependency at fault in the case of someone having bad goals, it is the goals at fault. So even the successful achievement of outrageous goals are not special cases of goal-*achievement*; they are special cases of goal-*having*. So the psychiatric issue in these cases, if there is one to be had, must have something to do with the goals the person has, not the rather mundane and predictable fact they use some tool (even if that tool is drugs) to achieve those goals. In making this shift, I show how we can both capture intuitive cases of SUD and solve puzzling cases.

Chapter 2

An example of a puzzling case we can answer under a teleological framing of dependence concerns adaptive preferences. Suppose a person claims to successfully depend on alcohol. That is, they have a goal, they use alcohol to achieve it, successfully achieve that goal, and accept all the consequences involved. As the previous chapter explains, the current model of SUD says this person does not have SUD so long as her goals remain acceptably achieved. In this chapter, I explain why that makes sense as a diagnostic standard for pathological use—rather than just reporting the state of the literature. In cases of alleged addiction, I argue that what is often a more valid concern is the alleged patient’s values. To make this point, I connect feminist philosophical literature (Khader 2011, Terlazzo 2018) on “adaptive preference” with the current addiction

literature, and argue: Under the current model of addiction, adaptive preferences are not addictions.

Chapter 3

What is not debated: addictions cause harm. Because addictions cause harm, addictive behavior is often considered irrational in the game-theoretic sense. Meaning: it does not make sense that one would continuously use something, like drugs or alcohol, that runs orthogonal to her preferences. It is this self-induced contradiction of preferences that philosophers and clinicians describe as “irrational.” While there is opposing literature that posits a theory of rational addiction, the word “rational” here always refers to agents with forward-looking and stable preferences. Using a simple game theoretic setup, I show how even in cases involving *unstable* preferences, rational choices do not prevent addiction.

Future Work

...